Starved Into Disability: Colonial Violence

The 1943 Bengal Famine,
and the Strategic Creation of a Disabled Bengal

Introduction

We often, as we were growing up in a South Asian household, heard our moms, grandmothers, or someone from our family say to avoid sugar without realizing there is a much deeper reason behind this than the average explanation of sugar being poisonous for a regular human body. This ties back to the Bengal Famine. In the spring of 1943, people began dying of starvation on the streets of Kolkata. They had walked from rural Bengal, hoping the city might offer survival. Most found nothing. By the time the famine ended, between two and three million people were dead (Dyson & Maharatna, 1991). Many more survived with lasting physical damage: neurological injuries from beriberi, severe wasting from kwashiorkor, and stunted development in children born during the crisis. Fast forward to today; the region now has one of the highest rates of type 2 diabetes in the world. The combination of genetic predisposition and modern lifestyle factors creates a perfect storm for the disease.

The irony is that South Asians, who were once starved by their colonizers, are now overnourished but still suffering from the metabolic consequences of those past famines. It's as if the body is still preparing for the next famine that may never come, leading to a greater risk of developing type 2 diabetes, heart disease, and other metabolic disorders. (Scholar's Quill, 2024)

None of this has been analyzed as a disability event. That silence is telling. It reflects a persistent tendency in disability studies to locate disability in individual bodies rather than in the political and economic systems that actively produce impairment. The Bengal Famine exposes this limitation directly. Disability was not incidental to the famine. It was one of its central outcomes, shaped by the same colonial hierarchies of race, class, gender, and caste that determined who lived and who died. This paper argues that the 1943 Bengal Famine is a paradigmatic case of the colonial production of disability and that analyzing it requires frameworks grounded in the intersections of race, colonialism, and structural violence that Western disability studies has been slow to develop.

Colonial Capitalism as a Disabling Force

The standard frameworks for disability studies in the West were not built to explain what happened in Bengal. The medical model treats disability as a problem residing in the individual body. The social model, as Oliver (1990) describes it, shifts attention toward social barriers, but it emerged from a specific Euro-American context and was not designed to account for colonialism as a disabling force. Kafer (2013) pushes further, arguing that disability cannot be theorized without attention to the political conditions under which certain bodies are made expendable. Even so, the field has been slow to take the Global South seriously as a site of theory. Meekosha (2011) puts it plainly: "Disability studies has largely ignored the global dimensions of disablement" and continues to reproduce a Northern-centric bias that makes invisible the mass disablement caused by "war, famine, environmental destruction, and imperial extraction. " The Bengal Famine is exactly the kind of event she is describing.

The famine was not a natural disaster. Churchill's wartime cabinet prioritized food exports from Bengal, refused to authorize relief shipments despite direct warnings from Secretary of State Leo Amery and Viceroy Wavell, and dismissed famine reports with open contempt. Churchill's recorded response to one such telegram—"Then why hasn't Gandhi died yet?" (quoted in Choudhury, 2021)—was not an aberration. It reflected a coherent colonial logic. Erevelles (2011) theorizes exactly this logic when she argues that the violent production of disability under colonialism and capitalism "is integral to the maintenance of existing social relations of production." The Bengali laboring poor were not accidentally disabled by a crisis the British failed to prevent. They were disabled by policies the British chose to implement. That distinction matters for how disability studies understands its own subject.

Baynton (2017) offers a complementary lens. He argues that disability has historically been used to justify the subjugation of entire groups: "The attribution of disability was a crucial step in justifying unequal treatment." In the colonial imagination, Bengali and Indian subjects were routinely constructed as racially inferior, physically and mentally incapable of self-governance. Churchill himself described Indians as "a beastly people with a beastly religion" who brought famine on themselves "for breeding like rabbits" (quoted in Choudhury, 2021). This rhetoric served a function. By naturalizing Indian bodies as inherently deficient, colonial ideology made the famine's disabled survivors invisible as political subjects. Their impairments were absorbed into a pre-existing story of racial inferiority rather than recognized as injuries caused by the state.

Intersectionality: Race, Gender, Caste, and Who Gets Disabled

The famine did not disable people randomly. It followed the existing fault lines of Bengali society and the colonial structures layered on top of them. Women and girls were disproportionately affected. Food was typically allocated first to men within households, and colonial relief work schemes required the kind of heavy physical labor that excluded most women from eligibility (Collingham, 2012). People who were already disabled before the famine—those who were blind, deaf, or physically impaired—had the least capacity to navigate collapsing markets or access food kitchens. Caste compounded all of this: lower-caste families had less land, less savings, and less social capital to draw on. Sarkar (2020) documents that among the three million who died, none were from the bhadralok (the educated, upper-caste Bengali middle class). The famine's disability was produced at the intersection of all these hierarchies at once.

Bailey and Mobley (2019) argue, in their Black feminist disability framework, that "race, gender, and disability are mutually constitutive systems of power and oppression." Their framework centers on the American context, but the analytical logic applies here. In Bengal, the production of disability through starvation and disease was organized by the same logic of intersecting hierarchies. Erevelles (2011) makes this structural dimension explicit: the bodies deemed killable or disposable in conditions of colonial occupation are "already located at the bottom of social hierarchies organized around race, gender, class, and (dis)ability." The British relief infrastructure confirmed this. It prioritized workers in war-related industries in Kolkata, who had access to subsidized food, while systematically excluding rural peasants, women, and the poor. Disability was concentrated in the populations already made most precarious by colonial rule.

Ineese-Nash (2020) argues that colonialism does not merely encounter disability, but it also actively constructs categories of ability and disability in order to serve administrative and ideological ends. She describes this process as "disability as a technology of colonization." In Bengal, the British administered bodies through military fitness classifications, census categories, and relief eligibility criteria that defined ability in terms of usefulness to the war effort. Famine survivors who emerged with permanent impairments were not recognized as people who had been disabled by colonial policy. Their conditions were treated as personal or racial failures. The colonial state that produced their disability simultaneously erased itself as the cause.

Disability Justice and the Question of Liberation

Disability studies, Clare (2017) writes, "must be shaped not by the quest for tenure and academic success but by the ferocious need for liberation." That is a challenge aimed directly at scholarship, like this paper. It is easy to produce a structural critique of the Bengal Famine—to trace colonial policy, cite mortality figures, and invoke the right theoretical frameworks. What is harder is to ask what any of this analysis is actually *for*. Clare answers that question clearly. Disability studies needs to be useful to "people disabled through war, genocide, and environmental destruction." The people who starved in Bengal in 1943, who survived with permanent neurological damage, and whose children were born into bodies marked by famine, are the people he is talking about. An analysis that stops at critique, without asking what justice would look like for them, has not yet answered Clare's challenge.

What would disability justice actually demand here? Berne (2016), drawing on the principles developed by Sins Invalid, points to a few things: leadership by those most impacted, cross-movement solidarity, and the refusal to treat disability as separate from race, colonialism, and economic inequality. Applied to the Bengal Famine, this framework exposes something uncomfortable. The communities most severely disabled by the famine had no say in post-famine relief policy. Their ways of knowing and surviving were not treated as relevant. The anti-colonial movements that emerged from Bengal in the 1940s were fighting for collective survival and bodily freedom, but those struggles were not legible to Western frameworks as disability justice, even though that is, in many ways, what they were. As Connell (2011) argues, the fights across the Global South against colonial exploitation are, at their core, fights over what happens to bodies .

Meekosha's (2011) argument is worth sitting with. She is not asking disability studies to add more case studies from the Global South to its existing reading lists. She is asking the field to recognize that "the epistemic diversity that exists globally" means the Global South is not a site for applying theory—it is a site for generating it . The Bengal Famine raises questions the existing literature has not answered: about how colonial states tracked and managed impaired bodies, about what survival looked like for disabled people when relief was structured around labor capacity, and about the long-term intergenerational effects of famine-produced disability on communities already living under occupation. These are not minor additions to a Northern framework. They are different questions altogether, and disability studies has largely not asked them.

Conclusion

Three million people died in Bengal in 1943. The famine that killed them was not a drought, not a crop failure, not a tragedy of circumstance. It was the result of decisions made by a colonial government that had calculated, clearly and deliberately, that Bengali lives were less important than wartime resource extraction. The millions who survived carried the physical consequences of that calculation in their bodies for the rest of their lives. Disability studies has, for the most part, not registered this. That absence should be read as a problem with the field, not a gap in the historical record.

What this paper has tried to show is that the Bengal Famine is not a marginal case study. It is a central example of how colonialism produces disability—how impairment is generated through the deliberate organization of race, labor, food, and survival. Erevelles (2011) gives us the framework to see this: disability production is not incidental to empire; it is built into it. Meekosha (2011) gives us the provocation: a disability study that cannot see this is still centering the wrong experiences. And Clare (2017) gives us the standard: the point of all of this is liberation, not publication. The Bengal Famine asks disability studies to take all three of those demands seriously at the same time. Whether the field rises to meet them is a different question—but it is the right one to be asking.

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